DPP DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	CVAD: MEDICATION ADMINISTRATION
DEPARTMENT:	INTENSIVE CARE UNIT
APPLIES TO:	ALL MEDICAL AND NURSING STAFF

CODE AND VERSION NUMBER	
DPP- ICU-0015(v6)	
LAST VERSION APPROVAL DATE:	18 / 07 / 2022
NUMBER OF PAGES:	4

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APPROVED DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
09 / 02 / 2025	23 / 02 / 2025	TRIENNIAL	08 / 02 / 2028

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1. PURPOSE

1.1. To provide procedural guidelines how to administer medications through Central Venous Access Devices appropriately.

2. DEFINITION & ABBREVIATION

2.1. **Central Venous Access Device (CVAD)** – also known as Central Line, Central Venous Line or Central Venous Catheter. It is a multipurpose venous catheter that terminates at or close to the heart or in one of the great vessels. It is placed into a large vein in the neck (Internal Jugular Vein), chest (Subclavian Vein or Axillary Vein), and groin (Femoral Vein).

2.2. **Medication Administration** – a continual process and goes beyond the task of simply giving medication to a client.

2.3. **Aseptic Technique** – a procedure used by medical staff to prevent the spread of infection.

2.4. **HH (Hand Hygiene)** – a general term that applies to routine hand washing, antiseptic hand wash, antiseptic hand rub, or surgical hand antisepsis.

2.5. **MCC**- Medica Cloud Care System (**HIS** – Hospital Information System)

3. RESPONSIBILITY

3.1. **ICU Physician** – Responsible to write the complete medication prescription legibly preferably in generic name by using only hospital's approved abbreviation.

3.2. **ICU Staff Nurse** – Responsible for administering medication within their scope of practice. Knowledgeable about the indication, side effects and interactions of medications and take actions as necessary.

3.3. **ICU Head Nurse** – Ensures that all staff are aware of this policy

4. POLICY

4.1. There must be a complete physician's order in in the MCC.

4.2. ICU Staff Nurses must be trained and competent in CVAD medication administration.

4.3. All intermittent drugs are to be flushed post administration to clear Y-port and smart site bung of residual drug.

4.4. Ensure to follow the 8 Rights of Medication Administration (Refer to MPP-MM-0015 Medication Administration).

4.5. Strict compliance with IPC guidelines in standard precautions when performing the procedure. (Refer to MPP-IPC-0001 IPC Guidelines in Standard Precautions).

5. PROCEDURE

5.1. Physician will write the order in MCC.

5.2. Identify the patient with 2 identifiers and date of birth. (Refer to APP-QPS-0016 Patient Identification).

5.3. Gather all sterile equipment pack needed.

5.4. Perform hand hygiene and don sterile gloves and gown.

5.5. Physician will explain the procedure to the patient if conscious and alert.

5.6. Establish whether the patient has any known allergy.

5.7. Provide privacy.

5.8. Ensure to follow hand hygiene protocols and wear gloves. (Refer to APPH-IPC-0005 Guidelines in Hand Hygiene).

5.9. Inspect the central catheter insertion site for signs of skin discoloration or infection.

5.10. Clean the CVAD port using alcohol swab for 15 seconds and allow it to dry.

5.11. Prepare medications using drawing up needles taking care to protect key parts.

5.12. Clamp any lumens not being accessed. NOTE: Do NOT clamp lines with running vasoactive infusion.

5.13. Attach syringe to needleless adaptor, if infusions not running withdraw blood first until there is blood back flow to verify, patency.

5.14. Administer the medication as ordered.

5.15. Flush line with 15-20mls sterile normal saline using a pulsatile action. Lock and clamp if disconnecting as per CVAD locks/flushes.

5.16. Disposal of used equipment safely and properly then perform hand hygiene.

5.17. Monitor for any suspected CVAD dislodgement, line-related infection or other complication.

5.18. Document and sign in the MCC.

6. MEASUREMENT

6.1. 100% of the medical and nursing staff are well oriented and adheres to this policy.

7. REFERENCES /CROSS- REFERENCES

7.1. https://www.rch.org.au/policy/public/Central_Venous_Access_Device_Management/

7.2. MPP-MM-0015 Medication Administration

7.3. APPH-IPC-0005 Guidelines in Hand Hygiene

7.4. APP-QPS-0016 Patient Identification

8. ATTACHMENT

8.1. N/A

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design

9.2. Modification to the definition, responsibility, policy and procedure contents.